

Our Ref: (T) TS/LA/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

Moot Lane, Downton

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

CE145 Level E, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS] [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses: Severe terminal ileal Crohn's disease.

I reviewed Jason in the GI Clinic today. He remains relatively well in himself but does describe nagging discomfort in his right iliac fossa with what he describes as a potential to blockage that never progresses. He is eating normally and maintaining his weight. He is maintained on azathioprine 50 milligrams daily and this has been dose optimised.

His MR [NAME] significant terminal ileal disease which is at risk of causing obstruction or fistula and therefore I think we should start him on [NAME]. I have talked to him carefully about the risks and given him some written information. He will have a full infection screen today together with routine bloods and I have asked for copies of these bloods to be sent through to you.

I will ask specialist nurse to arrange his [NAME] prescription and he understands that we will re-image his small bowel after 12 months of treatment to determine whether this will be needed in the long term. It is important that [NAME] is given in combination with azathioprine.

We will see him again in the clinic in approximately four months' time.

With best wishes

Yours sincerely

[PROVIDER-NAME]

Consultant Gastroenterologist

Copies to:

Sister [NAME]